

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Shingrix for the prevention of Shingles

Patient Group Direction, version 005, issued 11 September 2026. Supersedes Version 004, 10 September 2026.

Summary of the governing guidance for shingles vaccination: Green Book chapter 28a (Shingles), the JCVI advice of 2021 that moved the programme to Shingrix, and the Shingrix SmPC. NICE CKS 'Shingles' summarises the same programme.

Purpose of Vaccination

- To **prevent shingles (herpes zoster)** and its complications, particularly **postherpetic neuralgia (PHN)**.
- Risk of shingles increases with **age** and **weakened immunity**.

Vaccines Available

There are **two shingles vaccines** currently in use in the UK:

1. **Shingrix® (recombinant, non-live)**
 - **Preferred vaccine**, especially for immunocompromised individuals.
 - Suitable for people aged 50 years and older, or 18 and over if immunocompromised (the latter group is not covered by this PGD: refer).
 - Administered **intramuscularly**.
2. **Zostavax® (live attenuated)**
 - Only used **if Shingrix is contraindicated or unavailable**.
 - Not recommended for immunocompromised individuals.
 - Single **subcutaneous** dose for **those aged 70–79 years** (if previously eligible and missed).

Eligibility (UK Immunisation Schedule)

- **Routine vaccination:**
 - All individuals **aged 65 years**, starting from 1 September 2023.
 - A **catch-up programme** includes those turning 66 to 70 years between 1 September 2023 and 31 August 2028.
- **Immunocompromised individuals:**
 - Aged 18 years and over are eligible for Shingrix® under the Green Book and SmPC, but THIS PGD covers individuals aged 50 and over only; an immunosuppressed adult under 50 is referred.
 - Includes those with conditions such as leukaemia, HIV, or on immunosuppressive therapy.
- **Unvaccinated individuals aged 70–79** remain eligible under the original **Zostavax®** programme if not contraindicated.

Dosing Schedules

- **Shingrix®:**
 - **Two doses** given **2 to 6 months apart**.
- **Zostavax®:**
 - **Single dose only**.

Contraindications

- **Shingrix:**
 - Hypersensitivity to any component.
- **Zostavax (live vaccine):**
 - Immunosuppressed individuals.
 - Pregnancy (defer until after delivery).
 - Hypersensitivity to gelatin or neomycin (if present in the formulation).

Precautions

- Delay vaccination in cases of acute illness with fever.
- Not for treatment of acute shingles.
- Check vaccine history prior to administration.

Common Side Effects

- **Shingrix:** Localised pain, fatigue, headache, myalgia, fever.
- **Zostavax:** Injection site reactions, mild rash, headache.

Effectiveness

- **Shingrix:** Over **90% effective** at preventing shingles and PHN.
- **Zostavax:** Around **50–60% effective**, lower efficacy in older adults.

Documentation

- Record date, site, batch number, and brand of vaccine.
- Report any suspected adverse reactions to the **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Shingrix

for vaccination against

Shingles

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Shingrix is indicated for the prevention of herpes zoster (shingles) and herpes zoster-related postherpetic neuralgia in individuals aged 50 years and older.
Inclusion criteria	- Individuals aged 50 years or older. - Has not completed a two-dose course of Shingrix. Dose 2 may be given under this PGD where dose 1 was given elsewhere; record the date of dose 1. Previous Zostavax is not an exclusion. - No history of shingles in the past 12 months. - Informed consent obtained. - Patient is eligible under national immunisation guidelines.
Exclusion criteria	Hypersensitivity to any component of the vaccine. Pregnancy or breastfeeding (not routinely recommended).
Cautions	Counsel patient that systemic side effects are common and generally self-limiting. Allow appropriate spacing from other vaccines (e.g., COVID-19 or influenza) based on clinical judgement.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Shingrix
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection, preferably in the deltoid muscle.
Dose and frequency	0.5 mL dose given twice, with the second dose 2 to 6 months after the first.
Quantity to be administered and/or supplied	0.5 mL per dose; two-dose course required
Maximum or minimum treatment period	Two doses required, spaced 2–6 months apart.
Adverse effects	Localised pain, redness, swelling at injection site, fatigue, headache, myalgia, shivering, fever, gastrointestinal symptoms. Refer to SmPC for full details.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 28a Shingles (herpes zoster); JCVI advice on the shingles programme; Shingrix Summary of Product Characteristics, current version
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the

responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
11/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date
--------------------------------------	----------------------------------	-----------	------

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025



Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.



Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask.

Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references name Green Book chapter 28a, JCVI and the Shingrix SmPC as the governing sources.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Observation after vaccination, Cold chain and excursions, Disposal, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No PGD-level period of validity was stated.

Inclusion: 'no previous shingles vaccination' excluded every patient attending for dose 2 of the two-dose course the PGD authorises, and any Zostavax recipient who now needs Shingrix. Reworded.

Version: v004

Issued: 10 September 2026

Supersedes: Version 003, 10 September 2026

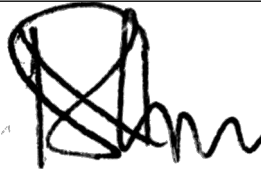
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Guidance summary: it stated that immunocompromised adults aged 18 and over are eligible for Shingrix, which the SmPC and Green Book support, while the PGD covers 50 and over only. The summary now says so. Whether to add an 18 to 49 immunosuppressed arm is recorded for decision.

Signed on behalf of Get Real Health

Version v005, 11 September 2026.

<i>Name</i>	<i>Qualification</i>	<i>Signature</i>	<i>Date</i>
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.